

ADC THERAPEUTICS SA (ADCT) ADCT

July 20, 2026 - 12:03pm EST

by [conway968](#)

			2026	2027
Price:	1.29	EPS	0	0
Shares Out. (in M):	185	P/E	0	0
Market Cap (in \$M):	240	P/FCF	0	0
Net Debt (in \$M):	-115	EBIT	-85	-55
TEV (in \$M):	125	TEV/EBIT	0	0

Description

ADC Therapeutics (NYSE: ADCT) is a commercial-stage biotech built around ZYNLONTA (loncastuximab tesirine), an approved CD19-directed antibody-drug conjugate (ADC). Zamperini previously wrote up ADCT at ~\$4/sh, but the stock has since cratered ~70% on the company's LOTIS-5 topline data. Although LOTIS-5 met its primary endpoint (PFS HR 0.73, p=0.008), the data were worse than expected. However, we believe the commercial impact of the Lotis 5 "miss" is minimal. Thus, the recent sell-off creates an incredible buying opportunity.

As a reminder, the ADCT thesis is that the existing 3rd line franchise provides a stable ~\$74mm revenue base which should grow 2-4x in the coming years with expanded uses. The expansion plan has three levers: LOTIS-5, LOTIS-7, and marginal zone lymphoma, of which LOTIS-5 is *least important*. We believe this thesis remains *intact*. At today's price, the market is attributing material probability to the drug being pulled from the market, which we view as highly unlikely.

LOTIS-5: Disappointing, but not Disastrous

In retrospect, expectations for LOTIS 5 were too high, and each of the shortcomings in the final data are attributable to difficulties presented by the trial design, which was FDA mandated:

SHORTCOMING	DESIGN DIFFICULTY
No OS benefit	Open label study , control arm moved to other therapies even before progressing. Imbalance of follow-on CAR-T.
Death imbalance + Safety	Patient deaths in drug arms are concentrated in elderly European patients, where supportive care seems deficient. Moreover, the measurable time on drug and time for AEs was longer in the LOTIS-5 arm .

The specific trial design shortcomings of LOTIS-5 reflect a broader tension between the sponsor's desire for low-risk trials and the FDA's demands for robust confirmatory evidence, and the result has been trials where the control arm gets the worst plausible regimen (R-GemOx in this case). Because the control is given on an open label basis, the treating physicians tend to switch these sick patients to better therapy ASAP. If this switch occurs prior to the patient failing the treatment, the patient is "censored" and moved out of the statistical analysis going forward, without ever having counted as a progressor.

Additionally, areas like Eastern Europe that lack good access to expensive therapies enroll the most patients. In the case of LOTIS-5, European physicians seem to have not used supportive care measures, such as prophylactic antibiotics and immunoglobulin, that lower the risk of infection from the known risk of cytopenias. As a result, a handful of elderly European patients died from infection, making both the PFS and OS for patients 75+ much worse than for the entire population.

These trial design shortcomings are inherent to the FDA requirements for this indication (DLBCL). They have caused even worse problems for competing drugs such as glofitamab (Roche), which was denied a label expansion because of drastically different outcomes across geographies in its STARGLO trial such that the FDA deemed the data inapplicable to the US population.

Why were investors expecting clean data? We believe that both management and investors were fooled into optimism by the clean safety profile of ZYNLONTA monotherapy and the excellent data from the 20-patient LOTIS-5 open label safety run-in, in which ZYNLONTA showed a 50% CR rate, median PFS of 8.3 months, and no safety problems. In this small run-in, the safety issues around the elderly and the need for supportive care did not emerge, and the lack of a control arm gave no hints of the heavy censoring.

Nonetheless, LOTIS-5 *was a success* with the primary PFS endpoint hit (HR 0.73, p=0.008). The PFS benefit incorporates the impact of

grade 5 AEs and is understated due to heavy censoring in the control arm. We believe it supports FDA approval of label expansion to second line in combination with rituximab, and that the data, particularly in the pre-specified population of < 75 years old, supports use of this fixed duration, “targeted chemo” regimen in the ~30% of 2L patients still being given systemic chemo. In the event the FDA disagrees, we still expect NCCN guideline support—the precedent is STARGLO—which drives adoption in community oncology, and we see no scenario where ZYNLONTA 3L+ is pulled from the market.

LOTIS-7: The Real Prize

The DLBCL treatment landscape is secularly shifting from systemic chemo-based regimens to superior targeted regimens, such as CAR-T and bispecifics. With off-the-shelf convenience, bispecifics are best positioned, and glofitamab (Roche / Genentech, “Columvi”) is a growing market leader.

We have always viewed LOTIS-7, which combines ZYNLONTA with glofitamab, as the most important driver of ZYNLONTA revenue expansion. The addition of ZYNLONTA to this already popular treatment appears to materially improve both efficacy and safety. Efficacy is improved through the combined targeting of CD20 (glofit) and CD19 (ZYN), which appears to increase response rates materially compared to glofit monotherapy. Safety appears to be improved through a lowering of CRS, a dangerous immune hyperreaction that is the most troubling side effect of glofit.

The LOTIS-7 trial recently completed enrollment of 100 patients and will read out top line data in 4Q. It is an open label trial, and we have already seen data on the first 49 patients (December 2025). These data show that the combination has a CR rate of 78%-- 50-100% better than that shown in glofit monotherapy-- and a low-grade CRS rate of ~25%-- roughly half of glofit monotherapy.

We believe this trial will lead to NCCN guideline inclusion, enabling reimbursement and establishing glofit + ZYN as the preferred 2L treatment regimen in DLBCL. If ZYN were to attach to even half of current glofit treatments, that would mean ~\$150mm of incremental revenue to ADCT.

Marginal Zone Lymphoma: A Low-risk Tack-on

Marginal Zone Lymphoma is a rare, indolent CD19-positive lymphoma with chronic, relapsing treatment paradigms. In an open label MZL study, ZYNLONTA is showing excellent results, with a CR rate of ~70%. When this study concludes, ZYN should receive NCCN inclusion and potentially a label expansion. While small, MZL has few good treatment options, so there is an opportunity for strong penetration. We believe MZL should ramp to \$30-50mm of high margin incremental revenue, beginning late 2027.

Compelling Valuation

We continue to believe that new revenue streams will begin layering on the ~\$70mm base business starting mid-2027, with minimal increase in opex. While LOTIS-5 creates uncertainty about how much ZYN will be combined with chemo, LOTIS-7 represents the better treatment that will take share from chemo in coming years. Assuming LOTIS-5 drives zero incremental revenue, we still see total ZYN revenues reaching \$300mm+ with 7 and MZL, which would produce \$150mm+ of EBIT and make the company worth \$900 or \$5 / share.

I do not hold a position with the issuer such as employment, directorship, or consultancy.

I and/or others I advise hold a material investment in the issuer's securities.

Catalyst

- August 2026: LOTIS-5 pre-sBLA meeting; the key signal on whether the FDA views the North American dataset as adequate.
- Q4 2026: Full LOTIS-5 dataset and sBLA submission + LOTIS-7 topline (~100 patients).
- 1H 2027: NCCN guideline / compendia decisions for LOTIS-5 and LOTIS-7 to facilitate 2L reimbursement; regulatory updates.
- Ongoing: strategic / value-maximizing action

Messages

Subject HCR Royalty Obligation
Entry 07/21/2026 10:55 AM
Member mack885

Conway,

Thanks for the writeup. How do you think about the \$750M owed on the \$325M royalty financing and the change of control payment amendment executed in February?

Thanks

Mack

Subject Re: HCR Royalty Obligation
Entry 07/23/2026 01:43 PM
Member conway968

Hi Mack,

We think of the HCR royalty as a royalty, not as debt. The rate scales from 7 to 10% over time. Zynlonta has 95% GMs. One can think of the royalty as taking that gross margin to 85%, which is still respectable.

Given that they advanced \$300mm, HCR can't be happy with royalties of \$6mm per year! Their hopes for a better return are either (a) growth in Zynlonta sales, as we project, or (b) a change of control. Prior to Feb 2026, the acquirer would have had to pay the full royalty up to the cap of \$750mm, which is a huge number. In Feb 2026, the company convinced HCR to lower the CoC upfront payment to \$150-200mm, and to let the royalty continue paying after the deal. This lower payment is more feasible.

The key is that, on a standalone basis, HCR is earning a terrible return of 2%, and that return will only improve to HSD even if ADCT crushes it with the expanded use cases. There seems an opportunity to again lower that CoC payment in order to make M&A attractive.

Subject Re: Re: HCR Royalty Obligation
Entry 07/23/2026 03:15 PM
Member mack885

Thanks conway. While the renegotiated buyout is an improvement on the original deal, it's still a very material cost (\$150-200M) relative to the \$160M the current market cap of ADCT.

How are you so confident that Zynlonta doesn't get withdrawn for the market on the large Grade 5 imbalance toward Zynlonta in confirmatory trial (13.2% vs. 4.6%) coupled with missing OS? Since it's only approved on an accelerated approval basis, the FDA will have to make a decision on full approval now that the confirmatory trial has read out. I understand that PFS was the primary but OS is still very material. Based on the Q&A from data call, the sell side is hyper focused on the Grade 5 imbalance. Do you think the Lotis-7 read out comes ahead of the sNDA and gives a shot at redemption prior to a potential drastic FDA move?

Side note- amazing how in biotech management gets to fail upwards on abysmal performance (see also KPTI's recent mgmt cash awards).

See the cash retention bonus awards from the 7/2/26 8-K:

On June 30, 2026, the Board of Directors of ADC Therapeutics SA (the "Company"), with the advice of its independent compensation consultant, approved one-time retention awards ("Awards") to certain of the Company's employees, including its named executive officers as set forth below:

Name	Title	Cash Incentive Award	RSU Award
Ameet Mallik	Chief Executive Officer	\$1,795,500	675,000
Jose Carmona	Chief Financial Officer	\$541,842	203,700
Mohamed Zaki	Chief Medical Officer	\$568,974	213,900

The Awards were granted pursuant to incentive award letter agreements (the "Incentive Award Agreements") that set forth the terms and conditions of the respective Awards.

The cash portion will be paid on or about July 15, 2026. The cash portion is subject to repayment if the named executive officer's employment with the Company terminates before June 30, 2027. Such repayment obligation will not apply if employment is terminated by the Company without cause or by the named executive officer for good reason.

Subject	Re: Re: Re: HCR Royalty Obligation
Entry	07/27/2026 12:00 PM
Member	conway968

Hi Mack,

Thanks for the thoughtful questions.

I agree that the \$150mm buyout is not ideal, but we'd expect a takeout scenario to be approaching \$1B. Since the CoC is credited against the royalty cap, a buyer should only value it as a large pull forward, worth perhaps \$75mm vs \$150mm.

If the company sells at a lower price, there is likely an opportunity to negotiate a second time with HCR. The status quo of earning \$6mm per year from a thinly capitalized stand-alone is really terrible for them.

I am as confident (as possible with the FDA) that the product will not be withdrawn and view the worst realistic case as negotiating/conducting a second confirmatory study. This is essentially what's happening with Glofit after their CRL, and it's worth noting EPCORE DLBCL-1 also missed its primary endpoint. Based on this precedent, outright withdrawal seems unlikely. It's also worth noting the bispecifics' NCCN inclusion despite not receiving full approval post-CRL.

Zynlonta's monotherapy safety has been manageable. While adding Rituximab caused more harm than physicians expected, the totality of the data suggests a positive benefit/risk in at least some patients (on PFS and OS too) and the headline safety data's applicability to the US isn't 1:1. The safety profile in LOTIS-7 thus far has been cleaner, and I believe up-to-date LOTIS-7 safety data will be submitted to FDA as part of the label expansion package.

Regarding timelines, we think they will get LOTIS-5/NDA clarity first in September. The LOTIS-7 data will likely come toward year-end (maybe around ASH; abstracts earlier) so they'll have regulatory visibility sooner, for better or worse.

I am not a fan of the retention package. The company argues that the next 12-18 months mean everything (LOTIS-7 readout, FDA label expansion, MZL readout), so they cannot afford to lose key team members.